BRIEFING NOTE

TO: Policy Director, Department of Health

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STATEMENT OF PURPOSE: Healthcare Worker Burnout in Canada: The Case for a National Wellbeing Strategy

ISSUE

Canada's healthcare system is facing huge crisis. Burnout among healthcare workers has reached emergency levels, and without action from the federal government, the country will keep losing the people its health system depends on. This briefing notes argues that the federal government needs to adopt a National Healthcare Worker Wellbeing Strategy to deals with the root causes of burnout, protect the workforce, and keeps quality care available to all Canadians.

BACKGROUND

Burnout is a work-related condition that includes emotional exhaustion, feeling disconnected from work, and a reduced sense of personal accomplishment (Ontario COVID-19 Science Advisory Table, 2021). In Canada, it has become one of the most serious threats to the healthcare workforce. The problem started before the pandemic. Even before COVID-19, between 20% and 40% of healthcare workers in Canada showed severe burnout (Ontario COVID-19 Science Advisory Table, 2021). The pandemic made it a lot worse. A 2024 cross-sectional study of over 2,000 Canadian public health workers found that 78.7% met the clinical criteria for burnout (Singh et al., 2024). Nurses had the highest burnout rate of any healthcare profession, reaching 89.5% during the pandemic, (Balakrishnar et al., 2025). In spring 2022, 266,900 healthcare workers across Canada were reported working overtime which is the highest number ever recorded (Petal Health, 2025).

The effects of this crisis are showing across the system. A 2025 national study found that 25% of healthcare workers planned to leave their jobs, and 44% of them said burnout was the main reason (Orr et al., 2025). In Quebec alone, about 43% of nurses leave the profession before turning 35 (Petal Health, 2025). Since 2019, Canadian emergency departments have been closed for a combined 47,500 days, and by 2024, one in five hospitals had experienced an unplanned shutdown (Petal Health, 2025).

Healthcare delivery in Canada falls primarily under provincial and territorial jurisdiction under the Canada Health Act and the Constitution Act. But the federal government has a clear responsibility to provide national coordination, workforce data, and provide funding. Canada does not have a unified national strategy for healthcare worker wellbeing. Bourgeault et al. (2019) describe this gap as costly, risky, and unfair. Without federal leadership, provinces are left to deal with the issue on their own/

POLICY OPTIONS & ANALYSIS

There are three main policy options the federal government could consider. Each one has its own strengths and limitations.

Option 1: Implement a National Healthcare Worker Wellbeing Strategy.

The Royal College of Physicians and Surgeons of Canada (2023) has already called for a Canada-wide plan to support healthcare worker health and wellbeing. This strategy would create national standards for tracking burnout, provide funding to provinces and territories, ensure access to mental health supports in healthcare workplaces, and set up accountability systems. Health Workforce Canada's Strategic Plan (2025) also supports this type of approach.

Pros: This is the most durable and long-term solution. It focuses on fixing the root causes of burnout instead of just dealing with the symptoms. It allows for better use of data for decision and it also shows the federal government is committed to supporting healthcare workers

Cons: It requires ongoing funding and strong coordination between federal and provincial governments, which has been difficult in the past (Bourgeault et al., 2019). It is also the slowest option to show short-term results.

Option 2: Legislate Workload Caps and Reduce Administrative Burden.

Heavy workloads are one of the main causes of burnout. Research shows that long hours, poor staffing ratios, and too much administrative work can all contribute to exhaustion and workers leaving their jobs (Balakrishnar et al., 2025, Austin & Crown, 2023). This option would involve federal government support and to encourage provinces to create safe staffing laws and invest in digital systems to reduce paperwork.

Pros: This option directly targets the main causes of burnout. Reducing administrative work can be done quickly using existing health information systems. There is strong evidence linking workload to burnout. (Balakrishnar et al., 2025)

Cons: Staffing laws are controlled by provinces, so the federal government cannot directly enforce them. Reducing working hours without increasing staff could shift the workload onto others. This option also does not address the mental health needs of workers who are already experiencing burnout.

Option 3: Mandate Peer Support and Mental Health Programs for Healthcare Workers.

This option would require hospitals and healthcare workplaces to provide free, confidential mental health services to all healthcare workers, including peer support programs and employee assistance programs. The (Canadian Nurses Association, 2025) highlights psychological safety as essential for a stable workforce, and research shows that these programs can help reduce burnout (Singh et al., 2024).

Pros: This option is less expensive compared to a national strategy and can be implemented quickly through existing occupational health frameworks. It focuses on the mental impact of burnout, which is often the most overlooked part of the problem.

Cons: It focuses on treating the symptoms rather than fixing the causes of burnout. Many healthcare workers do not use these services because of stigma (Ontario COVID-19 Science Advisory Table, 2021). Without changes to workload and working conditions, this option alone may not be able to stop workers from leaving.

CHALLENGES

Regardless of which option is chosen, there would be unavoidable challenges. The division of responsibilities between federal and provincial governments makes coordination difficult, and provinces may resist federal decisions that affect their control over the healthcare system (Bourgeault et al., 2019). Funding is another issue, especially since health budgets are already tight. Most importantly, burnout is closely connected to staff shortages.

As more workers leave, those who remain face heavier workloads, which makes burnout even worse (Austin & Crown, 2023). Any effective solution needs to address both staffing levels and working conditions at the same time.

RECOMMENDATIONS

The strongest option is Option 1, creating a National Healthcare Worker Wellbeing Strategy. The current approach, where each province handles the issue separately, has not worked (Bourgeault et al., 2019; Orr et al., 2025). A coordinated national plan, based on the framework proposed by the Royal College of Physicians and Surgeons of Canada (2023), would give provinces the tools and support they need to act effectively.

As a next step, the Department of Health should bring together federal, provincial, and territorial health ministers to begin working toward a national framework, with a goal of reaching an agreement within 18 months. While this is being developed, Option 3 should be used as a temporary measure to provide immediate support to workers already experiencing burnout. Canada cannot afford to keep losing healthcare workers. Without a healthy workforce, the healthcare system cannot function.

REFERENCES

Austin, Z., & Crown, N. (2023). Health human resources planning in Canada – Part I: Opportunities and challenges for pharmacy. Canadian Pharmacists Journal, 156(6), 310–316. https://doi.org/10.1177/17151635231201802

Balakrishnar, K., Long, B.-Z. S., Haritos, A. M., Formuli, E., & Nowrouzi-Kia, B. (2025). Factors associated with intent to leave and burnout among Canadian nurses amidst the COVID-19 pandemic. Healthcare Management Forum. https://doi.org/10.1177/08445621251338580

Bourgeault, I., Maier, C. B., Dieleman, M., Ball, J., MacKenzie, A., Nancarrow, S., Pallas, S. E., & Sidat, M. (2019). Poor health workforce planning is costly, risky and inequitable. Canadian Medical Association Journal, 191(42), E1147–E1151. https://doi.org/10.1503/cmaj.190948

Canadian Nurses Association. (2025). CNA's policy roadmap for 2025 and beyond. https://www.cna-aiic.ca/en/policy-advocacy/2025-federal-election-platform

Health Workforce Canada. (2025). Health Workforce Canada strategic plan 2025–2028. https://healthworkforce.ca/strategic-plan/

Ontario COVID-19 Science Advisory Table. (2021). Burnout in hospital-based healthcare workers during COVID-19. https://covid19-sciencetable.ca/sciencebrief/burnout-in-hospital-based-healthcare-workers-during-covid-19/

Orr, T., Cheung, E., Saha, M., Balogun, T., Feng, C., & Farag, M. (2025). Identifying risk factors for burnout-driven turnover in Canadian healthcare workers during the Covid-19 pandemic. BMC Health Services Research, 25, 469. https://doi.org/10.1186/s12913-025-12522-1

Petal Health. (2025, October 13). The cost of burnout: How Canada's healthcare shortage limits access to care. https://www.petal-health.com/en/clinics/the-cost-of-burnout-how-canadas-healthcare-shortage-limits-access-to-care/

Royal College of Physicians and Surgeons of Canada. (2023). Canada's health workforce well-being plan. https://www.royalcollege.ca/en/about/strategic-themes/health-workforce-wellbeing-plan.html

Singh, J., Poon, D. E. O., Alvarez, E., et al. (2024). Burnout among public health workers in Canada: A cross-sectional study. BMC Public Health, 24, 48. https://doi.org/10.1186/s12889-023-17572-w